



To whom it may concern,

The City of Coral Springs has reviewed CMS guidance on surveillance testing as a response to the COVID-19 public health crisis. We **are considering** ~~will be~~ conducting regular non-diagnostic pooled surveillance of city employees as part of our COVID-19 risk mitigation strategy. **Such pooled surveillance would be in groups of four employees and identifiable to those employees via a bar coded system.** We would appreciate your help in affirming our understanding of CMS's guidance on surveillance testing in the context of our program configuration.

Our understanding of the current CMS guidance is based on the [6-19-2020](#) FAQ, the CMS [guidance on university lab testing](#), and our conversations with surveillance testing program operators. We understand that "SARS-CoV-2 surveillance testing can be performed in a facility that is NOT CLIA certified, and may use a test or technique NOT authorized by the FDA, provided that patient specific results are not reported." In addition, we understand that "CMS is temporarily exercising enforcement discretion under CLIA for SARS-CoV-2 surveillance testing, specifically, neither CMS nor the State survey agencies on its behalf will cite non-CLIA certified facilities, provided that the facility does not report actual test results, but only refers an individual to a CLIA-certified laboratory for further testing." We further understand that no individuals can be given a patient specific result from any testing and that examples of results include but are not limited to a status of "negative, positive, inconclusive, presumptive positive, a result of clinical significance, or a result of potential clinical significance," and that "if at any time a patient-specific result is reported, the facility is subject to CLIA and required to obtain an appropriate CLIA certificate in accordance with 42 CFR part 493."

Coral Springs will process pooled surveillance tests of our employees **in groups of four** at least weekly. Program participants in pools where the laboratory detects SARS-CoV-2 will be referred to take a clinical diagnostic test. Conversely, program participants in pools that the laboratory fails to detect SARS-CoV-2 will not be referred to take any action. All participants will sign a consent acknowledging they understand the non-diagnostic nature of the surveillance testing. Participants **and surveillance groups** will not receive **any** individual clinical diagnostic results, and the following terms will not be used: "Negative, Positive, Inconclusive, Presumptive positive," **or any** ~~A~~ result of clinical significance, or a result of potential clinical significance. **Instead, the group of four would only be notified that they must report to a CLIA certified laboratory for further testing.**

We believe this surveillance testing strategy significantly reduces our city's COVID-19 risk. In parallel, we will utilize **FDA approved** diagnostic COVID-19 testing **administered by a CLIA certified laboratory** and additional interventional measures. Therefore we would appreciate affirmation that our understanding is consistent with CMS's most current guidance on surveillance testing.

Best,

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